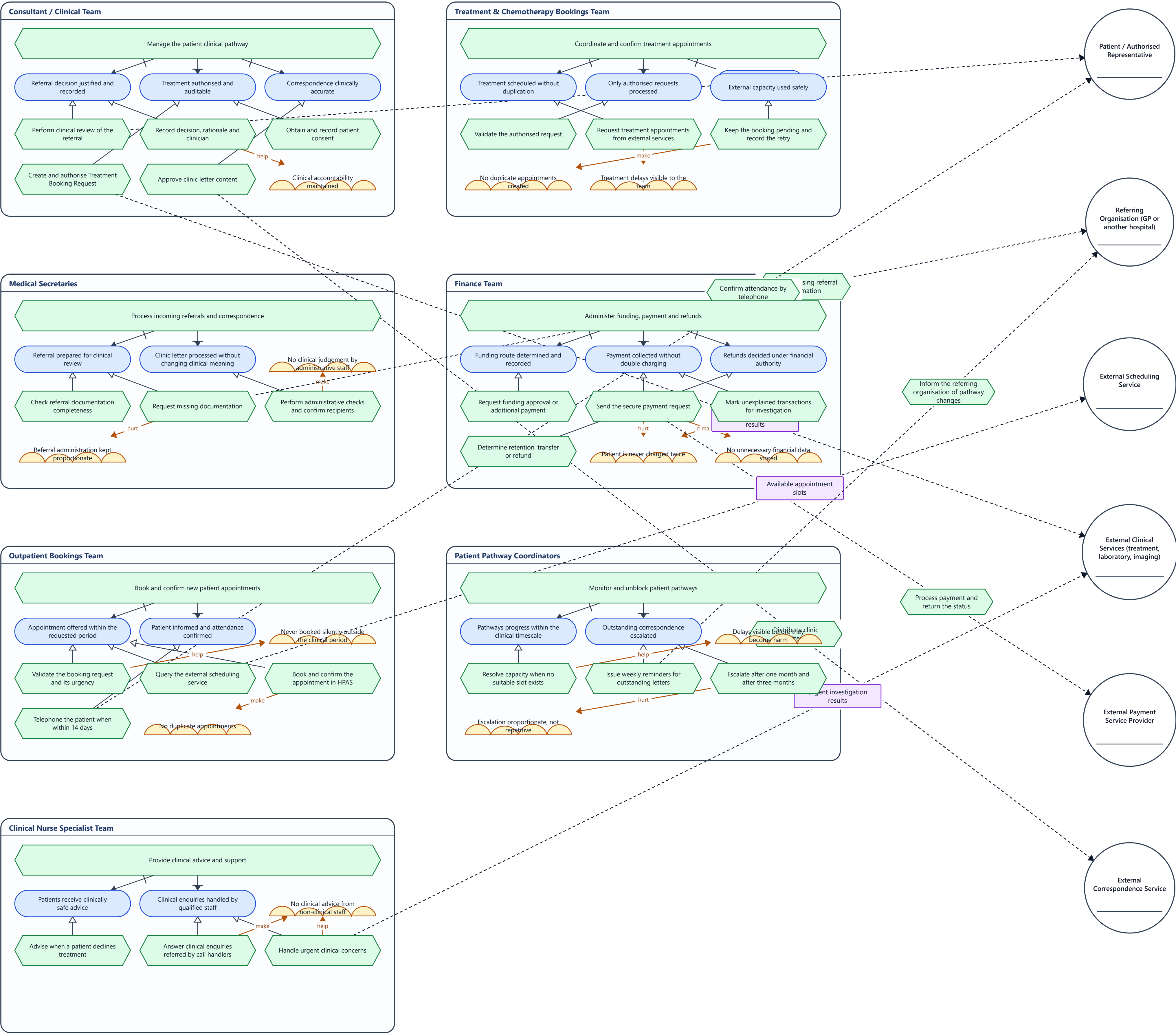




Legend and link notation

Assumptions, modelling choices and limitations - i* Strategic Rationale model

PURPOSE AND SCOPE

Seven actors are opened up: Consultant / Clinical Team, Medical Secretaries, Outpatient Bookings Team, Treatment and Chemotherapy Bookings Team, Finance Team, Patient Pathway Coordinators and Clinical Nurse Specialist Team. Between them they own every organisational relationship the case study describes: clinical authority, referral administration, appointment booking and the fourteen-day contact rule, treatment coordination, funding and payment, pathway monitoring and escalation, and clinical advice. Four further SD actors (the patient, the referring organisation and the external services) are modelled as dependees only, because the case study does not describe their internal rationale.

ASSUMPTIONS AND LIMITATIONS

An actor absent from the SR is out of scope for this rationale, not free of rationale. Contribution labels express qualitative influence only and carry no weights, so the trade-offs they expose are argued rather than measured. The models were produced after the initial release, so they evaluate the design rather than having shaped it.

NOTATION

Task decomposition carries the AND bar at the parent end: all components are needed. Means-end uses a hollow arrowhead at the goal end: the task is one way of achieving the end. Contribution links are labelled make, help, hurt or break and point at the soft goal they affect. Dashed links crossing an actor boundary are the dependencies drawn in the SD model.

KEY RATIONALE AND THE CONFLICT IT EXPOSES

Send the secure payment request contributes hurt to patient is never charged twice and make to no unnecessary financial data stored. The negative contribution is deliberate and is the model evidence for the process rule that a payment whose confirmation was lost is marked for investigation and never re-requested. Record decision, rationale and clinician contributes help to clinical accountability, and no administrative task has a means-end path to that soft goal, which is why clinical authority cannot be delegated.

CONSISTENCY WITH THE SD MODEL

Every actor opened here appears in the SD model; every dependency type used in the boundary links is used in the SD model; and each boundary link corresponds to a dependency drawn in the SD model. Where an SR actor has no internal means-end path to a dependency it has accepted, that dependency is an unmitigated risk, which is what the alignment evaluation uses to separate requirements that are supported from requirements that only appear to be.